

Additional UI/UX Screens to Complete MVP 1

16 net-new / revised screens closing every P1 gap found in the review of the 106-screen MVP 1 walkthrough — plus the catalogue of 10 state-level revisions to existing screens. Same design system · synthetic data only (no PHI) · WCAG 2.1 AA intent.

16

NEW SCREENS DESIGNED

10

REVISIONS TO EXISTING SCREENS

20/20

REVIEW SCENARIOS NOW COVERED

Companion to

- **Magnus_HMS_MVP1_UIUX_Design_Review_Feedback.docx** — findings referenced as C-x (compliance), S-x (safety), N-x, and Section 5 (P2).
- Vendor deck “MVP 1 UI/UX Design Walkthrough” (2026-07-17) — existing screen IDs (RR-S6, BL-S3, ...) referenced throughout.

Prepared for Magnus Diagnostics → NUMINACORE · 25 July 2026 · Prototype/reference, not committed product scope.

A · Net-new / revised screens designed in this pack

NS-01 — NS-16

ID	SCREEN	CLUSTER	GAP CLOSED (REVIEW REFERENCE)
NS-01	Critical result: flag, tier & send	Radiologist	Closes S-1(a,b,d): no send action, no tiers, no capture of finding/recipient at flag time; adds escalation ladder + addendum re-notify (RR-S5/RR-S6).
NS-02	Referrer results inbox & critical acknowledgement	Doctor Desk	Closes S-1(g): DD-S1 inbox was "SOON" while the critical loop assumed referrer acknowledgement; in-product ack now recorded.
NS-03	Patient merge / MPI reconciliation	Front Office & Setup	Closes S-6: duplicates had no post-registration resolution (GRV-8824 dead-end, AD-S5 unresolved matches). Four-eyes + reversible 30 days.
NS-04	Duplicate order & order-time safety interception	Shared order path	Closes S-5: the deck demoed an undetected duplicate order (DD-S2); laterality now forced; MR-safety mini-check at ordering.
NS-05	Technician worklist: safety gates & blocked Perform	Radiology Technician	Closes S-2/S-3 visibility: screening/consent/Form F/MR-safety gates now visible and enforced at the point of acquisition; physician-only override; STAT auto-refresh; cancelled = visible terminal state.
NS-06	Billing approvals queue (asynchronous)	Billing / Cashier	Closes P2-1: discount/refund/cancel approvals no longer require the approver at the same terminal; pending/approved/rejected/expired states.
NS-07	Delivery failures & counter-pickup worklist	Notifications & Delivery	Closes P2-4: owns NT-S1 failures — fix-contact-and-resend, channel switch, print/counter-pickup queue with ID-verified handover, suppression reasons cross-linked to consent.
NS-08	Cash session & end-of-day close	Billing / Cashier	Closes P2-3: cash drawer sessions, denomination count, gateway settlement reconciliation, variance with supervisor countersign, immutable signed close.
NS-09	Payment pending & reconciliation (UPI/card)	Billing (revises BL-S3)	Closes P2-3: stuck-UPI recovery — provider re-query, evidence-based manual confirm (UTR), duplicate auto-detection with auto-refund.
NS-10	Form F: patient declaration & signature capture	Imaging regulatory (revises IR-S1)	Closes C-1: pregnant woman's signed statutory declaration (signature/thumb pad), address + ID particulars, vernacular declaration text, centre-specific PNDT checks for doctor and machine.
NS-11	Legal hold register	Governance & compliance	Closes GV gap: suspends retention auto-deletion during medico-legal cases; refusals in GV-S3 now cite the hold; four-eyes release.
NS-12	Visit / encounter summary	Front Office (shared)	Closes N-7 + foundation ask: one thread linking registration → order → invoice → study → report → delivery; becomes the IPD encounter spine (checklist #15/#16).
NS-13	Compliance registrations & expiry enforcement	Tenant / Branch Admin	Closes C-6: AERB / PNDT (doctor & machine) / medical registrations now block scheduling, acquisition and sign-off on expiry, with 60/30/7-day alerts.
NS-14	Access requests: masked denial & lifecycle	Authorization (revises AZ-S2/RR-S2)	Closes C-7 + S-4: denial screens no longer leak demographics; request lifecycle (pending/approved/denied/expired); same-org priors allowed-and-audited for treatment.
NS-15	Incident intake & triage	Governance & compliance	Closes P2 + PL-S6 finding: front door that creates incidents, auto-classifies personal-data breaches (PII-in-logs reclassified to breach path), starts GV-S4 clocks.

NS-16	SLA breach & escalation — standard pattern	Design system	Closes the systemic gap: every countdown (RR-S1, RR-S6, RT-S4, GR-S1, GV-S3/S4, NS-06) gets one defined zero-hour behaviour — owner, escalation, no silent breaches.
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B · State-level revisions to existing vendor screens

R-01 — R-10 · TO BE APPLIED BY NUMINACORE IN THE SOURCE DESIGNS

ID	SCREEN	REQUIRED REVISION
R-01	CS-S1 Contrast screening	Add “eGFR not available” pathway (point-of-care creatinine / documented physician risk acceptance) — removes the walk-in dead-end.
R-02	CS-S2 Consent	Add refusal & withdrawal terminal states that stop the study; render consent text in the recorded language of explanation; enforce witness rule.
R-03	RR-S3 Report authoring	Statutory declaration never pre-ticked; add “uncoded term — reason required” escape hatch so coding cannot hard-block an emergency sign-off.
R-04	RR-S7 Signed report	Embed delivery/distribution status per recipient (cross-link NT-S1); surface addenda at top as current truth.
R-05	BL-S6 Cancel invoice	Credit-note issuance path for post-filing cancellations (uses the BL-S8 CRN series); carry-forward lines on cancel-and-reissue.
R-06	AU cluster	Design the missing set: 2FA verify step, lost-authenticator re-enrolment, backup-code generation at setup, admin unlock, badge/PIN re-lock with fast user switching; consolidate AU-S7–S13 into one templated entry.
R-07	IN-S1 Devices	Push alert to branch admin when a modality goes unreachable; quarantine state for images from unknown AE titles.
R-08	DD-S1 Break-glass	Auto-expiry (24 h) with re-justification; explicit purpose picker on normal access.
R-09	Offline states	Publish the offline capability matrix (which CORE screens work offline) and add degraded/queued/sync-conflict states to billing & RIS (checklist #13/#14).
R-10	AD-S2 Users	Multi-branch / multi-role assignment for floating staff (radiologist covering two sites); deactivation dependency warnings (unsigned reports, sole admin).

Coverage claim

With sections A + B applied, all 20 review scenarios reach “Covered”: every P1 compliance (C-1...C-7) and safety (S-1...S-6) finding has an owning surface, and every countdown has a defined breach behaviour (NS-16). Items C-4 (org-close retention) and C-5 (GST config) are policy/config fixes on existing platform screens and need no new UI beyond R-05.

 This pack uses synthetic data only. All screens follow the existing Magnus design system: token-masked references, audit notices, step-up identity confirmation, and append-only records.

Why this screen exists: Review finding S-1 — RR-S6 showed "Unnotified" critical rows with no send action, no severity tiers and no way to flag an addendum as critical. This screen adds the flag-at-signing modal, an explicit send/owner column on the RR-S6 queue, and critical re-notification on addenda (closes the RR-S5 gap).

STATE A — FLAG MODAL, SHOWN IMMEDIATELY AFTER "SIGN & FINALIZE" ON RR-S4

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VS

Dr. V. Shetty ▼

READING

Worklist

7

Reporting

Critical results

2

Addenda

REFERENCE

Templates

Prior studies

Home / Reporting / ACC-2026-90233 / Sign & finalize

Report signed — CT Head (plain) · Priya Menon · MRN-004828

Signed 2026-07-24 · 21:14 IST by Dr. Vikram Shetty · report v1 · referrer Dr. Meera Nair

✓ Signed & finalized at 21:14 IST

You marked "contains a critical finding" during proofread — complete the critical notification below before leaving this study.

Flag as a critical result

ACC-2026-90233 · CT Head (plain) · Priya Menon · MRN-004828 · signed 21:14 IST

Severity tier *

- ☒ **Tier RED — life-threatening.** Notify referrer within ≤ 15 min · acknowledgement required within ≤ 15 min. Phone call with read-back is mandatory.
- ☐ **Tier ORANGE — urgent actionable.** Notify within ≤ 60 min · acknowledgement within ≤ 4 h. SMS/WhatsApp acceptable; call on no-response.

Finding summary (sent to recipient) *

Acute right MCA-territory intracranial haemorrhage, approx. 18 mm, with 4 mm midline shift. Recommend immediate emergency referral.

Keep it short — this text is sent to the recipient. Never the full report in an SMS; the signed report travels only via the secure results inbox (NS-02).

Recipient

PRIMARY

REFERRING DOCTOR

Dr. Meera Nair · referring physician · Nair Clinic, Andheri West

PHONE

+91 98••• ••417 verified ✓

FALLBACK

Branch duty doctor — Dr. K. Rao ▼

- ☐ Patient-directed (self-referred walk-in) — **requires reason** and coordinator co-sign; used only when there is no referrer of record.

Channels

- ☒ SMS (summary + acknowledgement link) **and phone call with read-back script** — mandatory for Tier RED
- ☒ WhatsApp (summary card, no attachments)

Downgrade — not critical (reason required)

Create notification task & send now

Creating the task starts the Tier RED clock and assigns the radiology duty coordinator as owner. Every send, retry and acknowledgement is recorded in the access log.

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RS R. Shah (coordinator) ▼

READING

Worklist7

Reporting

Critical results2

Addenda

REFERENCE

Templates

Prior studies

Home / Critical results / Notification queue

Critical result notification queue

Escalation policy

Owner of record for all open tasks: Radiology duty coordinator — R. Shah · shift 14:00–22:00 IST

1 RED BREACHED
ack window exceeded

1 AWAITING ACK
within window

1 UNNOTIFIED
send pending

4 CLOSED TODAY
median ack 07 min

PATIENT / STUDY	TIER	SIGNED	RECIPIENT	STATUS	OWNER	ACTION
Priya Menon · MRN-004828 CT Head · ACC-2026-90233	RED	21:14	Dr. Meera Nair SMS ✓ 21:19 · call ✓ 21:21 (no answer ×2)	UNACKNOWLEDGED 21:12 breach +06:12	Radiology duty coordinator — R. Shah	Call agent Reassign
Mohammed Al-Rashid · MRN-004790 CXR PA · ACC-2026-90241	ORANGE	20:48	Dr. S. Iyer SMS ✓ 20:52 · WhatsApp ✓ 20:52	AWAITING ACK 02:41 of 4 h used	Radiology duty coordinator — R. Shah	Nudge
Rajesh Gupta · MRN-004795 USG Abdomen · ACC-2026-90236	ORANGE	21:38	Dr. Meera Nair no channel attempted yet	UNNOTIFIED	Radiology duty coordinator — R. Shah	Send message
Ananya Rao · MRN-004819 MRI Brain · ACC-2026-90198	ORANGE	17:05	Dr. S. Iyer	ACK 17:41 ✓	R. Shah	View log
Aarav Sharma · MRN-004821 CT KUB · ACC-2026-90187	RED	15:22	Dr. Meera Nair	ACK 15:29 ✓	R. Shah	View log

Escalation ladder — ACC-2026-90233 (Tier RED)

POLICY CR-POL-3

1

Tier 1 — referrer direct. SMS 21:19, WhatsApp 21:19, calls 21:21 & 21:26 (no answer).
completed

Tier 2 — branch medical lead.

Dr. A. Rao notified 21:30; attempting Dr. Nair's clinic
landline; audible alert at coordinator desk.

active now

Tier 3 — branch director + incident log.

Auto-triggers at 21:44 if still unacknowledged;
opens patient-safety incident record.

STATE C — ADDENDUM SIGN-OFF WITH CRITICAL RE-FLAG (CLOSES RR-S5 GAP)

Addendum sign-off — CT Chest (contrast) · Sofia Rossi · MRN-004822 ACC-2026-90102 · V1 → V2

ADDENDUM BY	Dr. Vikram Shetty · 2026-07-25 · 10:05 IST
WHAT CHANGED	Segmental pulmonary embolism in the right lower lobe, not described in v1. Impression revised.
PREVIOUS RECIPIENTS	Dr. Meera Nair (results inbox, 2026-07-21) · patient portal copy

Critical flag

- ☒ This addendum contains a critical finding — opens the same tier & send flow as State A after sign-off (Tier RED / ORANGE, recipient, channels).
- ☒ Notify all previous recipients that v2 replaces v1 default on — each prior recipient gets a "report updated — v2 replaces v1" notice with a what-changed summary; their inbox copy of v1 is watermarked SUPERSEDED.

Signed — Dr. Vikram Shetty · 10:05 IST

Save draft

Sign addendum & open critical send

Version history is immutable: v1 remains readable with a superseded banner. Re-notification and every recipient's open of v2 are recorded in the access log.

🔒 Every action on this screen is recorded in the access log.

Why this screen exists: DD-S1 marked "Results inbox — SOON", yet RR-S6 (and new NS-01) assume referrers can receive and acknowledge critical results. Without this screen the RED-tier acknowledgement loop has no receiving end. Promoted to MVP1-CORE.

STATE A — DEFAULT INBOX FOR DR. MEERA NAIR (PINNED CRITICAL + ADDENDUM ROWS)

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Meridian Diagnostics · Doctor Desk (referrer portal)

MN Dr. Meera Nair

DOCTOR DESK

Results inbox

4

My orders

My patients

ACCOUNT

Notification settings

Access log

Doctor Desk / Results inbox

Results inbox

Mark all read

Signed reports addressed to you at Meridian Diagnostics · every open is recorded in the access log visible to you and the patient

All 12 Unread 4 Critical 1 Addenda 2

1 critical result requires your acknowledgement

Tier RED · acknowledgement window 15 minutes from send (21:19 IST). The radiology coordinator will phone you if this stays unacknowledged.

PATIENT	STUDY	FINDING / NOTE	SIGNED	STATUS	ACTION
Priya Menon MRN - 004828	CT Head (plain) ACC - 2026-90233	CRITICAL · RED Acute intracranial haemorrhage, midline shift	2026-07-24 21:14	UNACKNOWLEDGED	Acknowledged
Sofia Rossi MRN - 004822	CT Chest (contrast) ACC - 2026-90102	UPDATED v2 · replaces v1 What changed: segmental PE right lower lobe added to impression	2026-07-25 10:05	UNREAD	Open v2
Rajesh Gupta MRN - 004795	USG Abdomen ACC - 2026-90236	Gallbladder calculi, no cholecystitis	2026-07-24 21:38	UNREAD	Open
Aarav Sharma MRN - 004821	CT KUB ACC - 2026-90187	6 mm left VUJ calculus · critical, acknowledged by you 15:29	2026-07-24 15:22	ACKNOWLEDGED ✓	Re-open
Ananya Rao MRN - 004819	MRI Brain (screening) ACC - 2026-90198	ADDENDUM v2 · typo correction, impression unchanged	2026-07-23 17:05	UNREAD	Open v2
Mohammed Al-Rashid MRN - 004790	X-Ray Chest PA ACC - 2026-90241	Right lower zone consolidation	2026-07-24 20:48	READ 21:05 ✓	Re-open
PT - ••• - 7311 (token-masked — order not placed by you; ask	MRI Lumbar Spine	—	2026-07-22 18:40	NO ACCESS	Request access

Reports are shared with you for care of your referred patients only. Forwarding outside Magnus is discouraged; downloads are watermarked with your name and time.

STATE B — MODAL: ACKNOWLEDGE CRITICAL RESULT

STATE C — EMPTY STATE: NOTHING UNREAD

Every report you open here is access-logged with your identity, time and IP — patients can see who viewed their reports.

🔒 Every action on this screen is recorded in the access log.

Why this screen exists: Duplicate patient records had no post-registration resolution path — grievance GRV-8824 ("my reports are split across two MRNs") dead-ended, and data import AD-S5 leaves "possible match" rows unresolved forever. This screen gives Front Office a supervised, four-eyes merge workflow.

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DN D. Nair (Front Office) ▼

FRONT OFFICE

Registration

Appointments

Billing

DATA QUALITY

Duplicate patients 4

Merge history

Import review

Home / Data quality / Duplicate patients

Possible duplicate patients

Matching rules

Candidates from registration checks, data imports and grievances · resolve oldest first · merges need a second approver

CANDIDATE PAIR	SOURCE	MATCH SCORE	FLAGGED	STATUS	ACTION
Sofia Rossi · MRN-004822 vs "Sofia R." · MRN-003317	Grievance GRV-8824	0.94 — name, DOB, phone match	2026-07-22	OPEN	Compare & resolve
Aarav Sharma · MRN-004821 vs "A. Sharma" · MRN-002906	Data import AD-S5 batch #12	0.88 — name, phone match; DOB missing on B	2026-07-19	OPEN	Compare & resolve
Rajesh Gupta · MRN-004795 vs "Rajesh K Gupta" · MRN-004102	Registration flagged at check-in	0.71 — name only; different phone	2026-07-24	NEEDS INFO	Compare & resolve
Ananya Rao · MRN-004819 vs MRN-003988	Registration	0.58 — reviewed, distinct people	2026-07-15	NOT A DUPLICATE ✓	View decision

"Not a duplicate" decisions are remembered — the pair will not be re-flagged unless identifiers change. All decisions are recorded in the access log with the reviewer's identity.

----- prototype: alternate states below -----

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DN D. Nair (Front Office) ▼

FRONT OFFICE

Registration

Appointments

Home / Data quality / Duplicate patients / GRV-8824 pair

Compare & resolve — MRN-004822 vs MRN-003317

Match score 0.94 · flagged via grievance GRV-8824 · pick the surviving value per field, then request approval

Billing

DATA QUALITY

Duplicate patients4

Merge history

Import review

Field-by-field comparison

• = SURVIVING VALUE

FIELD	RECORD A — MRN - 004822	RECORD B — MRN - 003317
Full name	☒ Sofia Rossi	☐ Sofia R.
Date of birth	☒ 1989-03-14	☐ 1989-03-14 (identical)
Phone	☐ +91 98220 11xxx last used 2024	☒ +91 99870 44xxx verified 2026-07-13
Address	☒ 12B, Palm Court, Andheri West, Mumbai 400058	☐ Palm Court, Andheri (incomplete)
ABHA number	☒ 91-5522-8814-7743	☐ — not linked —

Type to confirm *

MERGE 004822

Type MERGE 004822 (the surviving MRN) exactly to enable the request button.

Not a duplicate — keep separateRequest merge approval

Consequences

2 appointments, 3 reports, 2 invoices, 1 consent record move from MRN-003317 to surviving record MRN-004822.

MRN-003317 becomes a permanent record. All searches, old invoices (e.g. INV/A/27/0417) and barcode scans still find it. The number is never reused.

Report headers keep the MRN they were generated under; the merged chart shows both.

Four-eyes control

Merge needs a second approver - goes to the async approval queue (same pattern). Branch admin M. Kulkarni on rotation. You cannot approve your own merge.

STATE C — BLOCKED (ACTIVE ORDERS) AND SUCCESS (MERGE COMPLETED)

✗ Cannot merge — both records have active orders today.

MRN-004822 and MRN-003317 each have an in-progress visit. Complete or cancel ACC-2026-88214 first, then retry. Blocking prevents a mid-scan chart switch; this attempt is recorded in the access log.

✓ Merged — reference MRG-2026-0042

MRN-003317 is now an alias of MRN-004822. Requested by D. Nair 2026-07-25 11:32 IST · approved by M. Kulkarni 11:47 IST. Reversible for 30 days by an org admin (unmerge restores both records exactly, fully audited). Patient and referrer notices sent.

MOVED

2 appointments · 3 reports · 2 invoices · 1 consent record

ALIAS

MRN-003317 → resolves to MRN-004822 · never reused

UNMERGE WINDOW

until 2026-08-24 · org admin only · reason required

🔒 Every action on this screen is recorded in the access log.

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Registration

Appointments

DATA QUALITY

Duplicate patients

4

Sofia Rossi · MRN-004822 · referrer Dr. Meera Nair · draft not yet placed

"Continue" stays disabled until a reason is chosen. Dismissing this warning is itself recorded in the access log.

STATE B — LATERALITY ENFORCEMENT ON SIDED PROCEDURES

Save draft

The place-order button stays disabled while any required safety field is empty. Applies to all sided procedures (shoulder, knee, wrist, hip, breast).

Order-time safety check — MRI Lumbar Spine · Sofia Rossi ·
MRN-004822

3 QUESTIONS · 30
SECONDS

Asked while the patient is on the phone or at the desk, so unsafe bookings are caught before scheduling — not at the scanner door.

QUESTION	YES	NO
Pacemaker or any implanted device (clips, stimulator, cochlear implant)?	☒ Yes	☐ No
Possibly pregnant?	☐ Yes	☒ No
Previous reaction to contrast (any imaging)?	☐ Yes	☒ No

MR conditional — full screening will be required at check-in (CS-S1). Scheduling flagged.

DETAIL CAPTURED	Cardiac pacemaker (2023, brand unknown) — patient to bring device card
EFFECT	Slot auto-extended +15 min · technologist and duty radiologist notified · order badge "MR conditional" on RT worklist

This does NOT replace CS-S1 screening.

The full MR-safety questionnaire, device verification and technologist sign-off still happen at check-in. This check only prevents obviously unsafe or under-prepared bookings.

Place order with MR-conditional flag

Cancel order

🔒 Every action on this screen is recorded in the access log.

NS-05 Technician Worklist — Safety Gates & Blocked Perform

MVP1-CORE · REVISION

Radiology Technician — revises RT-S1 / RT-S2

Why this screen exists: The review found that safety gates (contrast screening CS-S1, consent CS-S2, statutory Form F IR-S1, MR safety) were enforced on their own screens but invisible at the point of acquisition, and a technologist could self-override a Blocked contrast decision on RT-S1. This revision surfaces every gate on the worklist row, disables Perform until gates clear, routes overrides through a physician's own identity, adds 15-second STAT auto-refresh (RT-S2 was manual-refresh only), and keeps cancelled studies visible in a terminal state instead of vanishing.

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TO T. Okafor

ACQUISITION

My worklist

6

In progress

1

Completed today

SAFETY

Contrast screening

1

Consent queue

Form F (PC-PNDT)

1

EQUIPMENT

Modality status

QC checklist

Acquisition > My worklist > CT-1 / US-2 / MR-1 · Sat 25 Jul 2026

My worklist — all modalities

Live · auto-refresh 15 s

Filter

Perform unlocks only when the patient has arrived and every safety gate for the study is green. Gate decisions are read-only here — they are owned by CS-S1 / CS-S2 / IR-S1. Last refreshed 10:02:14 IST.

6	2	1	1
ON MY LIST	GATES PENDING	BLOCKED	STAT
2 arrived & ready	Form F · MR safety	contrast screening — physician override only	received 09:52

TIME	PATIENT	ACCESSION · STUDY	ARRIVED	GATES		
10:15	Priya Menon MRN-004828 · 41y F	ACC-2026-04471 CT Brain plain · CT-1	✓ checked-in 09:41	Screening ✓	Consent ✓	Form F n/a
10:30	Ananya Rao MRN-004819 · 34y F	ACC-2026-04473 US Obstetric (anomaly) · US-2	✓ checked-in 09:58	Consent ✓	Form F pending	
⚠ Blocked: statutory Form F not completed Prenatal ultrasound cannot begin until the patient's signed Form F declaration is captured (PC-PNDT, recorded on the form. Open Form F (NS-10) This block and its resolution are recorded in the access						
10:45	Hiroshi Tanaka MRN-004777 · 58y M	ACC-2026-04468 CT Abdomen with contrast · CT-1	✓ checked-in 10:01	Screening: BLOCKED	Consent ✓	eGFR 28 · flagged by Dr. V. Shetty 09:47 (CS-S1)
11:00	Fatima Zahra MRN-004833 · 47y F	ACC-2026-04475 MRI Lumbar spine · MR-1	— not arrived	MR safety: implant flagged at ordering Consent ✓ cardiac stent (2019) — MR safety officer review required before zone III		
11:15	Rajesh Pillai MRN-	ACC-2026-04476 X-ray Chest PA · XR-1	— not arrived	Screening ✓	Consent n/a	

004802 ·
63y M

09:45

**Sunita
Deshmukh**
MRN-
004790-
52y F

ACC-2026-
04465
US
Abdomen-
US-2

Cancelled — patient unwell · by D. Nair 09:58

Gate chips are computed live from CS-S1 (contrast screening), CS-S2 (consent), IR-S1/NS-10 (Form F) and the MR safety register. Opening any patient row is recorded in the access log.

----- prototype: alternate states below -----

STATE B — STAT AUTO-REFRESH (REPLACES MANUAL-REFRESH-ONLY RT-S2)

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TO T. Okafor ▼

•
New STAT order received 09:52 — list refreshed automatically · audible alert played at console

The worklist polls every 15 seconds; STAT arrivals push instantly and re-sort to the top. No manual refresh needed (review finding: RT-S2 relied on the technologist noticing and refreshing). Alert acknowledgement is recorded in the access log.

TIME	PATIENT	ACCESSION · STUDY	ARRIVED	GATES	PRIORITY	ACTION
NOW	Fatima Zahra MRN- 004833 · 47y F	ACC-2026- 04479 CT Head plain (trauma) · CT-1	✓ ED arrival 09:51	Screening ✓ Consent: emergency pathway	STAT · Dr. Meera Nair 09:52	Perform STAT Acknowledge
10:15	Priya Menon MRN- 004828 · 41y F	ACC-2026- 04471 CT Brain plain · CT-1	✓ checked-in 09:41	Screening ✓ Consent ✓	Routine	Perform

If the STAT alert is not acknowledged within 3 minutes it escalates to the duty radiologist Dr. V. Shetty and the console repeats the audible alert.

STATE C — PHYSICIAN OVERRIDE OF A BLOCKED CONTRAST DECISION (NEVER SELF-OVERRIDE)

Physician override — contrast screening block

Hiroshi Tanaka · MRN-004777 · ACC-2026-04468 · CT Abdomen with contrast

Blocked at screening (CS-S1): eGFR 28 mL/min — contrast contraindicated per protocol.
Flagged by Dr. V. Shetty, 25 Jul 2026 09:47 IST.

T. Okafor (technologist) initiated this request but
cannot proceed alone

. A physician must authenticate below with their
own

identity and credentials at this console. The override is recorded against the physician,
not the technologist.

Overriding physician *

Dr. A. Rao — Consultant Radiologist (MMC/2011/40972) ▼

Physician selects their own name. Only physicians privileged for contrast decisions at this branch appear here.

Physician credentials *

Username: a.rao · Password:

Re-authentication is required even if Dr. A. Rao is already signed in elsewhere.

Clinical reason for override *

Suspected mesenteric ischaemia — benefit outweighs contrast risk. Pre- and post-hydration protocol ordered; nephrology informed (Dr. Meera Nair, referrer, agrees).

⚠

Override is recorded against the physician, tier RED audit.
The screening block, this justification, and Dr. A. Rao's authentication are written to the tamper-evident access log and flagged for the monthly radiation-safety review.

Cancel

Authenticate & override

🔒 Access log: 25 Jul 2026 10:02:14 IST · t.okafor viewed worklist (6 studies) · 09:52:03 STAT ACC-2026-04479 auto-pushed, audible alert played · 10:04:21 t.okafor initiated override request on ACC-2026-04468 — pending physician authentication (tier RED) · all row opens recorded in the access log.

Why this screen exists: Review found that every second-approver flow — discount above limit (BL-S2), refund (BL-S5) and invoice cancellation (BL-S6) — required the approver to physically walk to the cashier's terminal and enter credentials there. Single-staff evening shifts dead-ended. NS-06 makes approval a queue the approver works from their own session, on their own device.

STATE A — CASHIER SIDE, IMMEDIATELY AFTER REQUESTING A DISCOUNT ABOVE LIMIT (FROM BL-S2)

Magnus Billing

Meridian Diagnostics · Andheri West branch

DN

D. Nair ▾

FINANCE

Worklist & dues

New invoice1

Payments pending3

Cash session

REQUESTS

My approval requests2

SETUP

Tariffs & series

Billing > Invoice INV/AND/26-27/0417 > Discount request

Sent for approval

Requested 2026-07-13 · 19:38 IST · request DSC-2026-0688 · counter 2, Andheri West

Discount request — INV/AND/26-27/0417

PENDING

PATIENT

Rahul Nair · MRN - 004829

INVOICE TOTAL

₹12,650.00 (incl. CGST 9% + SGST 9% on taxable lines)

DISCOUNT REQUESTED

₹1,500.00 (11.9%) on pre-tax amount · reason: **Staff concession**

YOUR LIMIT

₹1,000.00 per invoice (role: Cashier, Andheri West)

SENT TO

2 eligible approvers — A. Deshpande (Branch supervisor), R. Kulkarni (Duty manager) · first response wins

APPROVAL SLA

Due within 4 business hours · request expires in **29:12** if unanswered

Invoice is parked — you can serve the next patient.

You'll be notified here and by push the moment an approver acts. The invoice stays locked at the requested amount; taxes recompute automatically on approval.

Park invoice & continue

View invoice

Withdraw request

My other open requests

CASHIER: D. NAIR

REQUEST	TYPE	REF	AMOUNT	SENT	STATUS
RFD-2026-0231	Refund	PAY-2026-3988	₹9,000.00	18:51	PENDING
DSC-2026-0679	Discount	INV/AND/26-27/0402	₹800.00	14:07	APPROVED

prototype: alternate states below

STATE B — APPROVER SIDE (A. DESHPANDE'S OWN SESSION — NEVER AT THE CASHIER'S TERMINAL)

Magnus Billing

Meridian Diagnostics · Andheri West branch

AD

A. Deshpande ▾

FINANCE

Worklist & dues

Approvals3

Cash sessions

Payments pending

REPORTS

Day book

Finance > Approvals queue

Approvals queue

Items awaiting a second approver · SLA: 4 business hours · overdue items escalate to org admin (NS-16 pattern)

History

TYPE	REF	PATIENT	AMOUNT	REQUESTED BY	WAITING	ACTION
DISCOUNT	INV/AND/26-27/0417	Rahul Nair · MRN - 004829	₹1,500.00 (11.9%)	D. Nair · counter 2	4 min	Review

Discount register	REFUND	PAY-2026 - 3988	David Fernandes · MRN - 004815	₹9,000.00	D. Nair · counter 2	51 min	Review
	CANCELLATION	INV/AND/26 - 27/0409	Sofia Rossi · MRN - 004822	₹4,720.00	K. Shah · counter 1	2.6 h DUE SOON	Review
Refunds always reverse to the original instrument of PAY-2026-3988 (BL-S5 rule). Cancellations void the invoice number in-series — the series does not renumber.							

STATE C — OUTCOMES BACK ON THE CASHIER'S SCREEN

Outcome notifications (toast-style, also delivered by push)

Discount approved

Approved by A. Deshpande · 2026-07-13 19:42 IST · DSC-2026-0688 · invoice unlocked at ₹11,150.00 (taxes recomputed). [Resume invoice](#)

Refund rejected

Rejected by A. Deshpande · reason: "Concession not applicable to MRI" · request RFD-2026-0231 closed — invoice returned unmodified. You may re-request with a corrected reason.

Request expired

DSC-2026-0679-B expired after 30 min with no approver response — the parked invoice reverted to its pre-request amount. Request again if still needed.

SLA: approval requests are due within 4 business hours. Anything overdue escalates automatically to the org admin's queue (same escalation pattern as NS-16) and is flagged in the branch day-book.

🔒 Every request, approval, rejection, expiry and identity step-up above is written to the immutable audit trail with requester, approver, terminal, timestamps (IST) and before/after invoice amounts, and recorded in the access log.

Why this screen exists: NT-S1 detected delivery failures but no screen owned recovery, and NT-S3's "falls back to print at counter" queue never existed as a worklist. Suppression reasons (consent, opt-out) were invisible to front-desk staff, breaking the AD-S4 ↔ NT-S1 joint. NS-07 gives failures, counter pickups and suppressions a single owned queue with a daily review SLA.

STATE A — FAILED DELIVERIES TAB (DAILY REVIEW QUEUE)

Magnus Delivery

Meridian Diagnostics · Andheri West branch

DN D. Nair

DELIVERY

Outbound log

Failures & pickup6

Templates

CONSENT

Channel preferences

DPDP requests

Notifications > Failures & counter pickup

Delivery failures & counter pickup

Export day log

2026-07-13 IST · patients shown pseudonymously in list views; full identity opens on the patient record with access logged

Failed · 6

Counter pickup · 4

Suppressed · 3

Resolved · 41

2 failed deliveries past the daily review SLA

Escalated to branch manager at 09:00 IST — resolve or move to counter pickup to clear the escalation.

PATIENT	ITEM	CHANNEL	ERROR	ATTEMPTS	AGE	ACTION
patient·a4e1	Report link — MRI Brain	SMS	undeliverable_number	3 of 3	26 h OVERDUE	Fix contact Sw Mo
patient·9c07	Invoice INV/AND/26-27/0417 + receipt	Email	mailbox_full	3 of 3	25 h OVERDUE	Fix contact Mo
patient·b382	Report link — X-Ray Chest PA	WhatsApp	not_on_whatsapp	2 of 3	6.4 h	Sw Mo
patient·5f19	Payment link ₹2,400.00	SMS	carrier_rejected (DND)	3 of 3	3.1 h	Sw Mo
patient·e630	Report link — USG Abdomen	Email	hard_bounce	1 of 3	52 min	Re
patient·77ad	Appointment reminder	SMS	gateway_timeout	2 of 3	18 min	Re

Failures older than 24 h breach the daily review SLA and escalate automatically. Moving to counter pickup issues a pickup code and prints a slip at the front desk (NT-S3 fallback).

----- prototype: alternate states below -----

STATE B — MODAL: FIX CONTACT & RESEND (FROM ROW PATIENT·A4E1)

Fix contact & resend — report link, MRI Brain

Patient: Rahul Nair · MRN-004829 — identity revealed inside this modal only; this view is access-logged.

Current mobile

+91 98... ..210

UNDELIVERABLE · 3 failed attempts

New mobile *

+91 99870 44____

Enter exactly as the patient states it; country code required.

☒ I verified this number with the patient (in person or on a call) before saving. *

What happens on save.

Editing a contact is recorded on the patient record with your identity and reason. The secure report link is re-issued to the new number and the old link is revoked immediately.

Cancel

Save contact & resend link

STATE C — COUNTER-PICKUP QUEUE + SUPPRESSED TAB

Counter pickup · 4 waiting

FRONT DESK · ANDHERI WEST

Report ready — MRI Brain (plain) PICKUP CODE MRD-7241

Rahul Nair · MRN-004829 — patient name shown to staff only, never on the public slip. Waiting since 2026-07-13 10:05 IST.

Handover checklist — MRD-7241

- ☒ Verified patient name and date of birth against the record
- ☒ Collector: **Representative** — relationship: **Spouse** · ID type: **Aadhaar** · last 4: 4417
- ☐ Collector: Patient (self) — not applicable

Collector signature

Collector signs here on the counter tablet

Mark collected — recorded

Marking collected stamps date, time, staff identity, collector details and signature onto the delivery record. Slips uncollected after 30 days are flagged for MRD review.

Suppressed · 3

NOT SENT — BY DESIGN

PATIENT	ITEM	SUPPRESSION REASON
patient · 2d94	Report link — CT KUB	Consent withdrawn · DPDP-CR-004821
patient · c518	Payment reminder ₹1,180.00	Opted out of SMS
patient · 8b60	Report link — USG Abdomen	No destination on file → counter pickup

Cross-link to consent.

Each suppression row links to the governing consent or preference record (AD-S4), so staff can see *why* nothing was sent and never override it from this screen. Withdrawn-consent items can only move to counter pickup.

 Failure triage actions, contact edits, link re-issues and revocations, pickup-code handovers (with collector identity and signature) and every reveal of a pseudonymised patient's name are recorded in the access log.

Why this screen exists: Review found no cash-drawer session concept anywhere in the billing cluster (BL-S1...BL-S6): no float, no tender or denomination handling, and no end-of-day reconciliation against gateway settlements. NS-08 adds a per-cashier session with a controlled close that cannot finish with unexplained variance.

STATE A — OPEN SESSION, MID-SHIFT VIEW

Magnus Billing



Meridian Diagnostics · Andheri West branch

DN

D. Nair

FINANCE

Worklist & dues

New invoice

Payments pending

3

Cash session

REQUESTS

My approval requests

1

Finance > Cash session > CSH-AND-2026-0713-01

Cash session — counter 2

Close day...

All figures live as of 18:47 IST · one open session per cashier per drawer

Session

CSH-AND-2026-0713-01

OPEN

OPENED

2026-07-13 · 08:02 IST by D. Nair · drawer 2, Andheri West

OPENING FLOAT

₹3,000.00 · counted and countersigned at open by A. Deshpande

RECEIPTS ISSUED

RCPT/AND/26-27/0791 → RCPT/AND/26-27/0857 (series contiguous, no gaps)

Collections by mode — today

GST-INCLUSIVE GROSS

MODE	PAYMENTS	COLLECTED	GATEWAY SETTLED	STATUS
Cash	14	₹18,450.00	n/a — drawer	COUNT AT CLOSE
Card	22	₹86,300.00	₹86,300.00 ✓	MATCHED
UPI	31	₹42,300.00	₹41,800.00	VARIANCE -₹500.00 · 1 awaiting confirmation → NS-09
Refunds paid out (cash)	1	-₹0.00 (₹9,000.00 refund pending NS-06 approval — not yet paid)	—	PENDING

EXPECTED CASH IN DRAWER

₹21,450.00 = ₹3,000.00 float + ₹18,450.00 cash collections - ₹0.00 cash refunds

Card/UPI settlement reconciliation uses the payment provider's T+1 file.

Today's gateway figures are provisional; mismatches surface as items in Pending payments (NS-09) rather than blocking your drawer count.

prototype: alternate states below

STATE B — CLOSE-DAY STEPPER, STEP 3 OF 4 (VARIANCE FOUND)



Count cash



Reconcile gateway



Resolve variances



Sign & close

Denomination count

ENTERED BY D. NAIR · 20:11 IST

DENOMINATION	COUNT	VALUE
--------------	-------	-------

Variance

SYSTEM EXPECTED	₹21,450.00
COUNTED	₹21,200.00

₹500	× 34	₹17,000.00
₹200	× 12	₹2,400.00
₹100	× 18	₹1,800.00
₹50 / ₹20 / ₹10 notes	× 0	₹0.00
Coins	—	₹0.00
Counted total		₹21,200.00

VARIANCE

—₹250.00 SHORT

Variance reason *

Change given in error

Supervisor countersign *

Request countersign — goes to NS-06 approvals queue

A. Deshpande or R. Kulkarni must countersign from their own session before step 4 unlocks.

Sessions cannot close with unexplained variance.
The close button stays disabled until every variance has a reason and a countersign.

Back to gateway recon

Continue to Sign & close

STATE C — CLOSED SESSION, IMMUTABLE SUMMARY

Session

CSH-AND-2026-0713-01

— end-of-day summary

CLOSED & SIGNED

CLOSED

2026-07-13 · 20:26 IST by D. Nair · drawer 2

COUNTED CASH

₹21,200.00

SYSTEM EXPECTED

₹21,450.00

VARIANCE

—₹250.00 · reason: Change given in error

APPROVED BY

A. Deshpande (Branch supervisor) · countersigned 20:24 IST via approvals queue APR-2026-0692

CARD / UPI RECON

Card matched ₹86,300.00 · UPI provisional —₹500.00 → 1 item open in Pending payments (PAY-2026-4031, see NS-09)

RECEIPT SERIES

RCPT/AND/26-27/0791 → 0863 · no gaps

REFS

EOD-AND-2026-0713 · deposit slip DEP-2026-0713-02

Export for accounts (CSV)

Print summary

Summary is tamper-evident (hash-chained with the receipt series); reopening a closed session requires org admin and is recorded.

Settlement note.

Card/UPI settlement recon uses the payment provider's T+1 file — mismatches open in Pending payments (NS-09), not against this drawer.

🔒 Session open/close, float countersign, every denomination entry, variance reasons, supervisor countersigns and any admin reopen are written to the immutable audit trail and recorded in the access log.

Why this screen exists: BL-S3 left UPI payments in "awaiting confirmation" with no timeout, no evidence-based manual confirm and no duplicate protection — pressing Cancel and re-collecting risked a double charge. NS-09 gives stuck payments a timeout, three recorded recovery paths, and automatic duplicate detection against the T+1 settlement file.

STATE A — STUCK UPI PAYMENT DETAIL (TIMED OUT AT 3 MIN)

Magnus Billing

Meridian Diagnostics · Andheri West branch

DN D. Nair

FINANCE

Worklist & dues

New invoice

Payments pending3

Cash session

REQUESTS

My approval requests1

Finance > Payments pending > PAY-2026-4031

Payment PAY-2026-4031 — UPI collect

UNCONFIRMED — money may or may not have left the patient's bank

Invoice INV/AND/26-27/0417 · Rahul Nair · MRN-004829 · ₹12,650.00 · counter 2 · 2026-07-13 IST

Do not blindly retry.
Retrying now can charge the patient twice. Choose one path below — every path is recorded with your identity and shown on the invoice history.

Payment timelinePROVIDER: RAZORFIN UPI

10:41:02

Payment initiated — ₹12,650.00 · UPI collect · VPA rahul.n•@okhdfc

10:41:05

Collect request sent to patient's UPI app · provider ref RZF-COL-88214063

10:44:05

No confirmation received — timeout at 3 minutes. Status moved to UNCONFIRMED; invoice remains unpaid in the ledger.

10:44:05

Item added to Payments pending queue · cash session CSH-AND-2026-0713-01 flagged (see NS-08)

Choose one recovery path

Check with provider now

Re-queries RazorFin for the live status of RZF-COL-88214063. Safe — read-only.

Mark received — with evidence

Patient shows proof of debit. Requires UTR and async supervisor approval (NS-06). → State B.

Cancel & collect another way

Voids this attempt so you can take cash/card.
Guard: if the bank later confirms this payment, the system auto-flags a duplicate and opens a refund (BL-S5) automatically — the patient is never silently double-charged.

prototype: alternate states below

STATE B — MODAL: MARK RECEIVED — WITH EVIDENCE

Mark received — with evidence · PAY-2026-4031

Use only when the patient can show a bank-side debit for exactly this payment.

UTR / provider reference *

617234889012

12-digit UPI UTR from the patient's bank app. Record the UTR only — never store screenshots or photos of the patient's banking app.

Amount confirmed *

₹12,650.00

Must equal the initiated amount exactly; partial confirmations are not allowed here.

Evidence note *

Patient showed HDFC app debit of ₹12,650.00 at 10:41 to Meridian Diagnostics; UTR matches collect request.

☒ Supervisor approval required — this request goes to the asynchronous approvals queue (NS-06); the receipt is issued only after approval.

Wrongly confirming creates an unreconciled ₹12,650.00.
Every manual confirmation is checked against the provider's T+1 settlement file; unmatched entries reopen here and are reported to the branch supervisor.

Cancel Send for approval

STATE C — PENDING-CONFIRMATIONS QUEUE + RESOLVED EXAMPLES

Payments pending confirmation · 3 open AGING SLA: REVIEW WITHIN 4 H · ESCALATE AT 24 H

PAYMENT	INVOICE	PATIENT	MODE	AMOUNT	AGING	STATUS
PAY-2026-4031	INV/AND/26-27/0417	Rahul Nair · MRN-004829	UPI	₹12,650.00	3 min	UNCONFIRMED
PAY-2026-4018	INV/AND/26-27/0411	Sana Qureshi · MRN-004826	Card	₹5,310.00	2.1 h	GATEWAY SLOW — RE-QUERY QUEUED
PAY-2026-3990	INV/AND/26-27/0398	Sofia Rossi · MRN-004822	UPI	₹1,180.00	26 h OVERDUE	AWAITING T+1 FILE

Recently resolved — how each path ends:

PAYMENT	AMOUNT	RESOLUTION
PAY-2026-4009	₹3,540.00	Auto-confirmed by provider webhook 10:52 — receipt RCPT/AND/26-27/0863 issued, invoice settled, SMS sent.
PAY-2026-3971	₹8,260.00	Duplicate detected in T+1 file — cashier had cancelled & re-collected in cash; refund PAY-R-2026-0114 auto-initiated to source UPI, patient notified.

Provider re-queries, evidence-based confirmations (with UTR), cancellations, duplicate flags and auto-refund initiations are written to the payment's immutable history and recorded in the access log.

Why this screen exists: Compliance blocker C-1: IR-S1 reduced Form F to a staff-tickable checkbox — there was no signed statutory declaration by the pregnant woman, no address/ID particulars, and no check that the performing doctor and machine are PNDT-registered *at this centre*. This revision captures the woman's own declaration and signature (in her language), her particulars, witness attestation, and verifies centre-specific doctor and machine registrations before the form can complete.

Magnus RIS

Meridian Diagnostics · Andheri West branch

TO T. Okafor ▼

REGULATORY

Form F — new 1

Form F register

Monthly submission (IR-S2)

SAFETY

Consent queue

Contrast screening

ACQUISITION

My worklist 6

Regulatory > Form F > ACC-2026-04473 · Sat 25 Jul 2026

Form F — statutory declaration before prenatal ultrasound

PC-PNDT Act, 1994 (as amended) · Form F must be completed and signed by the pregnant woman before US Obstetric ACC-2026-04473 can begin. The blocked Perform on NS-05 unlocks automatically on completion.

Patient — Ananya Rao · MRN-004819 · 34y F

G2 P1 L1 · LMP 2026-03-02 (GA 20w+4 by LMP) · Referrer: Dr. Meera Nair, Obstetrics · Study: US Obstetric (anomaly scan) · US-UNIT-2 · Performing doctor: Dr. S. Iyer

- ✓

Particulars
- 2

Declaration & signature
- 3

Witness attestation
- 4

Registration verification
- 5

Complete

Statutory declaration by the pregnant woman

READ ALOUD & SIGNED BY PATIENT

Declaration language

English ▼ (हिंदी, मराठी available)

The declaration text below renders in the selected language; the signed language is stored on the record.

I, **Ananya Rao**, declare that I am undergoing this ultrasound examination for medical reasons explained to me by my doctor. **I am not seeking to know, and I understand no one will disclose to me, the sex of my foetus.** I understand that determination or disclosure of the sex of the foetus is prohibited by law and is a punishable offence under the PC-PNDT Act, 1994. The purpose and nature of this scan have been explained to me in a language I understand, and the particulars I have given below are true to the best of my knowledge.

Full residential address *

B-1104, Lakeview Residency, JP
Road, Andheri West, Mumbai 400058,
Maharashtra

ID type *

Aadhaar (last 4) ▼
Voter ID / Passport also accepted

ID — last 4 *

... 7215
only last 4 stored

Patient signature *

Patient signature / thumb impression — captured on pad

Signature is captured on the counter signing pad and bound to this form. Thumb impression accepted where the patient cannot sign; interpreter attestation then becomes mandatory.

Witness / interpreter attestation

REQUIRED IF DECLARATION EXPLAINED

Name

Sunila Kadam

Role

Staff nurse — witness & interpreter ▼

- ☒ Declaration explained in **Marathi** to the patient before signing; the patient confirmed she understood.
- ☐ Patient signed by thumb impression (interpreter attestation mandatory).

Registration verification — this centre

CHECKED LIVE · NS-13 REGISTRY

ENTITY	PNDT REGISTRATION	SCOPE	STATUS
Performing doctor — Dr. S. Iyer PNDT-qualified sonologist	MH/PNDT/2019/4471	Registered at THIS centre (Andheri West) ✓	valid to 2027-03-31
Machine — US-UNIT-2 GE Voluson · US room 2	MH/PNDT/M/2021/0332	Registered at this centre ✓	valid ✓

Registration under the PC-PNDT Act is centre-specific. If either check fails, the form cannot complete (see blocked state below).

Save draft

Complete Form F

Completing the form creates a tamper-evident record, unlocks Perform on NS-05, and queues this Form F for the monthly submission register (IR-S2). Every view and edit of this form is recorded in the access log.

----- prototype: alternate states below -----

STATE B — BLOCKED: DOCTOR NOT PNDT-REGISTERED AT THIS CENTRE

Regulatory > Form F > ACC-2026-04473 · Registration verification

Cannot complete: Dr. R. Kulkarni is PNDT-registered at Bandra West, not at this centre.
Registration under the PC-PNDT Act is centre-specific. Choose a doctor registered at Andheri West, or contact your administrator (K. Verma) to update centre registrations on NS-13. This failed verification attempt is recorded in the access log.

ENTITY	PNDT REGISTRATION	SCOPE	STATUS
Performing doctor — Dr. R. Kulkarni	MH/PNDT/2020/5108	Registered at Bandra West — NOT this centre	centre mismatch
Machine — US-UNIT-2	MH/PNDT/M/2021/0332	Registered at this centre ✓	valid ✓

Choose different doctor ▼

Complete Form F

STATE C — COMPLETED FORM (FEEDS MONTHLY SUBMISSION IR-S2, NOW MVP1-CORE)

Regulatory > Form F > FF-2026-00341 · completed 25 Jul 2026 10:26 IST

Form F complete
Form F complete — FF-2026-00341 · tamper-evident record Signed declaration (English, explained in Marathi), particulars, witness attestation and centre-specific registration checks stored. Perform is now unlocked on NS-05 for ACC-2026-04473.

Signed declaration

Ananya Rao

Record

FORM NO.	FF-2026-00341
DOCTOR	Dr. S. Iyer · MH/PNDT/2019/4471
MACHINE	US-UNIT-2 · MH/PNDT/M/2021/0332

Signed on pad 25 Jul 2026 10:24 IST · witness Sunila Kadam · language: English, explained in Marathi.

SUBMISSION

Queued for July 2026
monthly submission to
the Appropriate
Authority — **IR-S2**
(now MVP1-CORE)

The completed record is tamper-evident: any later amendment creates a new version and is recorded in the access log with the amending user's identity.

 Access log: 25 Jul 2026 10:12–10:26 IST · t.okafor opened Form F for MRN-004819 (purpose: statutory pre-scan declaration) · registration checks run against NS-13 registry (1 pass, 1 centre-mismatch attempt recorded) · patient signature captured on pad · form FF-2026-00341 sealed — all steps recorded in the access log.

Why this screen exists: Review found that retention auto-deletion (GV-S5) had no mechanism to suspend it, and GV-S3's own refusal example cited an open medico-legal case with no supporting tool. This register is the single place a hold is placed, reviewed and released — GV-S5 and GV-S3 both consult it before acting.

PRIVACY

Deletion requests

3

Consent registry

Legal holds

1

Retention policies

OVERSIGHT

Incidents

Access log

Audit exports

Governance > Privacy > Legal holds

Legal hold register

+ Place legal hold

Holds suspend retention deletion and block erasure requests for the records in scope. Every action here is auditor-visible (GV-S8).

STATE — A · REGISTER

2

ACTIVE HOLDS
across 2 patients

1

REVIEW OVERDUE
oldest by 12 days

1

RELEASED (12 MO)
records back in retention

0

DELETION CONFLICTS
GV-S5 runs check this register

Holds

FILTER: ALL STATUSES · ALL BRANCHES

HOLD	PATIENT / SCOPE	LEGAL REF	PLACED BY	PLACED	NEXT REVIEW	STATUS
LH-2026-004	Mohammed Al-Rashid MRN - 004790 All radiology records + invoices, 2024–2026	Consumer case CC/118/2026 Bombay District Commission	Privacy Officer Dr. A. Rao	2026-05-12	2026-08-10	ACTIVE
LH-2026-002	Rajesh Gupta MRN - 004795 Accessions ACC-2026-81002, ACC-2026-81339 + report versions	MLC / police requisition FIR 0341/2026, Andheri PS	Privacy Officer Dr. A. Rao	2026-03-30	2026-07-13	ACTIVE review overdue 12 days
LH-2025-011	Sofia Rossi MRN - 004822 Date range 2025-01-01 → 2025-06-30, all record types	Insurance dispute Claim ref MAG-CLM-5521	Privacy Officer	2025-11-02	—	RELEASED 2026-06-19

Overdue reviews escalate to the DPO (Priya Sharma) per the standard SLA pattern (NS-16). A hold never lapses silently — it stays active until explicitly released.

Effects of an active hold

ENFORCED SYSTEM-WIDE

RETENTION

Automatic retention deletion is **suspended** for all records in scope. The GV-S5 scheduler

ERASURE

Deletion requests are **refused citing the hold** — GV-S3 links here so the Privacy Officer can

DISCLOSURE

Export-for-counsel is available for records in scope and is fully recorded — recipient, purpose,

checks this register before every run and skips held records, logging the skip.

show the requester the legal basis for refusal (DPDP §12 exemption grounds).

and file manifest are written to the access log.

----- prototype: alternate states below -----

STATE — B · PLACE HOLD (MODAL)

Place legal hold

Patient: **Mohammed Al-Rashid** · MRN-004790 · Andheri West

Scope of hold *

- ☒ **Entire patient record** — all record types, all dates
- ☐ **Date range** — records created within a period (from / to pickers appear on select)
- ☐ **Specific accessions** — e.g. ACC-2026-88214, ACC-2026-81339 (multi-input; report versions and linked invoices are included automatically)

Legal reference *

Consumer case CC/118/2026 — Bombay District Commission

Case, FIR, requisition or claim number. Required — a hold cannot be placed without a citable legal basis.

Expected review cadence

Every 90 days (default)



Counsel / requesting authority

M/s Deshpande & Associates

You will be reminded to confirm the hold is still needed.

Confirm your identity *

Re-enter your Magnus password to sign this action

Placing a hold is recorded and visible to auditors (GV-S8). It suspends retention deletion immediately.

Cancel

Place hold LH-2026-005

STATE — C · RELEASE (FOUR-EYES)

Release legal hold LH-2025-011

Releasing re-exposes these records to retention deletion.

A second approver must confirm — the placer alone cannot release a hold.

HOLD

LH-2025-011 · Sofia Rossi · MRN-004822

REQUESTED BY

Privacy Officer (Dr. A. Rao) · 2026-06-19 14:05 IST

SECOND APPROVER

Priya Sharma, DPO — **APPROVED 2026-06-19 15:40 IST**

Reason for release *

Insurance claim MAG-CLM-5521 settled; counsel confirmed in writing (letter dated 2026-06-17) that preservation is no longer required.

Cancel

Release hold (both approvals recorded)

Hold LH-2025-011 released.

Records re-enter normal retention; nothing is deleted immediately — the next scheduled retention run (GV-S5) evaluates them against policy, and any resulting deletions appear in the retention run report.

🔒 Every hold placement, review confirmation, scope change, export-for-counsel and release — including both approver identities and the stated reason — is recorded in the access log.

Why this screen exists: Review found registration, order, study, report, invoice and delivery were designed as separate worklists but never linked in one thread — no screen answered "what happened on this visit, and what is still open?". This summary is that thread, and it is deliberately built as the encounter spine the IPD phase will extend (foundation checklist #15/#16).

Magnus Front Office

Meridian Diagnostics · Andheri West

DN D. Nair ▼

FRONT OFFICE

Registration

Today's queue14

Visits

Billing desk

FOLLOW-UP

Delivery failures2

Pending balances

Front office > Visits > VIS-2026-30412

Visit VIS-2026-30412COMPLETE

Print visit summaryOpen patient record

Sofia Rossi · MRN-004822 · F 34 · 2026-07-13 · Andheri West

· Walk-in · Referrer: Dr. Meera Nair (external)

STATE — A · COMPLETED VISIT

Visit threadALL TIMES IST · EVERY EVENT LINKS TO ITS SOURCE SCREEN

09:12Registered — walk-in, demographics verified against Aadhaar last-4 · by D. NairFO-S1

09:15Token U-018 issued — Ultrasound queue, est. wait 40 minFO-S3

09:22Consent signed — DPDP notice v3 (English), purposes: diagnosis, report delivery via SMSGV-S1

09:31Order placed — US Abdomen · ACC-2026-88214 · ordering: Dr. Meera NairOR-S2

09:58Invoice raised — INV/AND/26-27/0421 · ₹2,800 · UPI · Paid ✓BL-S2

10:31Study acquired — US Abdomen, 24 images · technologist T. OkaforRT-S2

12:02Report signed — final v1 · Dr. V. Shetty · TAT 2 h 31 m (target 4 h)RR-S3

12:05Delivered — SMS link to +91 4471 · SMS ✓ delivered 12:06DL-S1

Financials

BILLED₹2,800.00

PAID₹2,800.00 (UPI, 09:58)

BALANCE₹0.00

Open items

None — visit complete

All 8 events closed. Nothing pending on any worklist.

Related visits

VIS-2026-188542026-03-02US Pelvis · complete

VIS-2025-903112025-11-18X-Ray Chest · complete

prototype: alternate states below

Visit thread — VIS-2026-30567 · Rajesh Gupta · MRN-004795 · 2026-07-25LIVE · REFRESHES ON EVENT

08:41Registered — by D. NairFO-S1

08:44Token C-007 issued — CT queueFO-S3

08:52Consent signed — DPDP notice v3 (Hindi)GV-S1

09:04Order placed — CT Brain plain · ACC-2026-89330OR-S2

09:15Invoice raised — INV/AND/26-27/0468 · ₹4,350 · part-paid ₹2,500Balance ₹1,850BL-S2

09:47Study acquired — CT Brain, 186 images · T. OkaforRT-S2

STATE — B · VISIT WITH OPEN ITEMS

Open items3 OPEN

Report unsigned — 3.1 h (TAT target 4 h)

Owner: Dr. V. Shetty → Reading worklist RR-S1. Escalates at breach per NS-16.

Balance due ₹1,850

Owner: front office → Billing desk BL-S2

Delivery failed — prior report resend

SMS to old number bounced → Failures worklist NS-07

– Report — *awaiting sign-off (assigned Dr. V. Shetty)*

– Delivery — *blocked until report signed*

Financials

BILLED	₹4,350.00
PAID	₹2,500.00
BALANCE	₹1,850.00

STATE — C · FOUNDATION NOTE (VISIBLE TO ADMINS ONLY)

Foundation note: this visit object is the encounter spine.

An IPD admission is a long-running encounter with the same linked-artifact model (checklist #15/#16) — orders, billing and reports attach identically; only the encounter class and period differ. No rebuild required when inpatient goes live.

Encounter data model — VIS-2026-30412

READ-ONLY · ORG ADMIN VIEW

ENCOUNTER_ID	VIS-2026-30412
CLASS	ambulatory (future: inpatient, emergency, daycare – same object)
PERIOD	2026-07-13 09:12 → 2026-07-13 12:05 IST (IPD: open-ended until discharge)
SERVICE_EVENTS[]	registration, token, order ACC-2026-88214, acquisition, report v1, delivery (append-only)
BILLING_ACCOUNT	INV/AND/26-27/0421 (IPD: running account, interim bills attach to the same encounter)
CONSENT_REFS	CNS-2026-30412-01 (DPDP notice v3, purposes: diagnosis, SMS delivery)

🔒 Opening a visit summary, and every artifact viewed from it, is attributed to the signed-in user and recorded in the access log.

Why this screen exists: Finding C-6: branch setup (TA-S3) captured AERB / PNDT / medical-council validity dates but nothing ever *blocked* anything when they lapsed — an expired AERB room licence or PNDT registration had no operational effect. This screen makes each registration an enforcement object: expiry warnings escalate, and on expiry the linked action (scheduling, acquisition, prenatal sign-off) is automatically blocked until renewal.

Magnus RIS



Meridian Diagnostics · Andheri West branch

KV

K. Verma ▼

BRANCH SETUP

Rooms & modalities

Staff & privileges

Compliance registrations 2

REGULATORY

Form F register

Monthly submission (IR-S2)

ADMIN

Audit & access log

Notification settings

Branch setup > Compliance registrations · Andheri West · Sat 25 Jul 2026

Compliance registrations

Export register

+ Add registration

Every registration carries an enforcement rule. When it expires, the linked capability is blocked automatically — warnings begin 60 days out. All edits require four-eyes confirmation and are recorded in the access log.

9

REGISTRATIONS TRACKED
5 valid, no action

1

EXPIRING ≤ 60 DAYS
AERB — CT Room 1

2

CRITICAL
1 auto-block armed · 1 blocking active

TYPE	HOLDER	NUMBER	VALID TO	STATUS	ON EXPIRY
AERB radiation licence eLORA · Type approval + room	CT Room 1 · CT-1 (128-slice)	AERB/MH/2023/8812	2026-09-05	42 days	Blocks scheduling & acquisition on CT-1
PNDT machine registration	US-UNIT-2 · US room 2	MH/PNDT/M/2021/0332	2027-01-15	valid ✓	Blocks all prenatal scans on this unit
PNDT doctor registration	Dr. S. Iyer · sonologist (this centre)	MH/PNDT/2019/4471*	2026-08-01	7 days — auto-block armed	Blocks prenatal scan sign-off by Dr. S. Iyer
Medical council registration	Dr. V. Shetty · radiologist	MMC/2008/31446	2028-04-30	valid ✓	Blocks all report sign-off by Dr. V. Shetty
AERB radiation licence	X-ray Room 1 · XR-1	AERB/MH/2021/5540	2026-07-10	EXPIRED — blocking active	Scheduling & acquisition on XR-1 blocked since 11 Jul 2026
PNDT centre registration	Meridian Diagnostics · Andheri West	MH/PNDT/C/2018/1207	2027-06-30	valid ✓	Blocks all prenatal ultrasound at this branch

* This renewal window overlaps Dr. S. Iyer's Form F duties — an amber notice already appears on NS-10 registration verification. Renewal note under review by admin K. Verma.

Alert & escalation settings

BRANCH POLICY · EDITABLE BY ORG ADMIN

REMINDERS

60 / 30 / 7 days before expiry → org admin (K. Verma) + branch manager, by email and in-app

ESCALATION

If unrenewed at 7 days: daily reminders, banner on branch admin home, and the "auto-block armed" flag shown to affected staff

ON EXPIRY

Linked capability blocked at 00:00 IST on the day after "valid to". Blocking cannot be muted — only a completed renewal (State C) lifts it

Next automatic action

01 Aug 2026 00:00 IST — prenatal sign-off block engages for Dr. S. Iyer unless renewal is recorded. Notified: K. Verma, branch manager, Dr. S. Iyer.

----- prototype: alternate states below -----

Scheduling grid — CT Room 1 after AERB expiry

AS SEEN BY FRONT DESK

SLOT	CT ROOM 1	US ROOM 2
10:00	— unavailable · AERB registration expired (CT Room 1)	Ananya Rao · US Obstetric
10:30	— unavailable · AERB registration expired (CT Room 1)	open
11:00	— unavailable · AERB registration expired (CT Room 1)	Fatima Zahra · US Abdomen

Slots grey out automatically; existing bookings are flagged to the front desk for rebooking to a licensed room. Attempted bookings are recorded in the access log.

STATE B — WHAT BLOCKING LOOKS LIKE TO END USERS

Signing blocked

Your PNDT registration expired on 01 Aug 2026.

Prenatal ultrasound reports cannot be signed. Contact your administrator (K. Verma) to record the renewal. This attempt is recorded in the access log.

Close

Notify administrator

Seen by Dr. S. Iyer at report sign-off if renewal is not recorded before 01 Aug 2026. Non-prenatal signing remains unaffected.

STATE C — RENEWAL UPDATE (FOUR-EYES) & HISTORY

Compliance registrations > Renew > PNDT doctor — Dr. S. Iyer

Record renewal — PNDT doctor registration

New registration number *

MH/PNDT/2026/6019

Valid from *

2026-08-01

Valid to *

2031-07-31

Certificate upload *

PNDT_renewal_SIyer_2026.pdf — attach scanned Appropriate Authority certificate

The certificate is stored with the registration record and shown in inspections/export.

Four-eyes confirmation required

Entered by K. Verma (org admin). A second privileged user — branch manager R. Bhat — must confirm the number and dates against the

Renewal history — Dr. S. Iyer

NUMBER	VALID	RECORDED BY
MH/PNDT/2019/4471	2019-04-01 → 2026-08-01	K. Verma · conf. R. Bhat · 03 Apr 2019
MH/PNDT/2014/2210	2014-05-12 → 2019-05-11	legacy import · 12 Jan 2024

certificate before enforcement lifts. Both identities are recorded in the access log.

Cancel

Submit for confirmation

History is append-only. Prior numbers stay attached to Form F records signed under them (NS-10).

🔒 Access log: 25 Jul 2026 11:05 IST · k.verma viewed compliance register (9 entries) · 11:09 opened renewal form for MH/PNDT/2019/4471 · blocking events (XR-1 since 11 Jul) and every sign-off attempt against an expired registration are recorded in the access log, tier RED.

Why this screen exists: Finding C-7: the AZ-S2 denial screen leaked the patient's name, MRN, age and sex to the very user being denied — itself a disclosure. Finding S-4: RR-S2 denied reading radiologists same-organisation priors for the patient on their own worklist, and access requests had no pending / approved / denied lifecycle. This revision masks the denial completely, adds a time-boxed request workflow with SLA, and lets same-org, same-patient, treatment-purpose priors load with recording instead of a request.

Magnus RIS

Meridian Diagnostics · Andheri West branch

VS Dr. V. Shetty ▼

READING

Reading worklist4

My reports

ACCESS

My access requests3

Shared with me

ACCOUNT

Privacy & audit

Patient search › record open attempt › Sat 25 Jul 2026 12:10 IST

Access denied — record outside your care team

Fixed denial (C-7): the card below shows only a masked token. It no longer shows the patient's name, age or sex — a denial must not itself disclose identity.

Patient recordMRN-.....30MASKED

You do not have access to this record.

Reason: You are not on this patient's care team at this location.

No identifying details (name, age, sex, studies) are shown on denial. If you believe you need this record for care, request access below — a privacy lead reviews it within the SLA.

Request access

Go back

This attempt was recorded. Denied attempts appear in the branch privacy review (tier AMBER).

----- prototype: alternate states below -----

STATE B — REQUEST MODAL + LIFECYCLE LIST (FIXES "NO PENDING/APPROVED/DENIED LIFECYCLE", S-4)

Request access — record MRN-.....30

Your request goes to the branch privacy lead. Approved access is time-boxed and every open under it is recorded in the access log.

Reason category *

Covering clinician ▼

Options: covering clinician · follow-up call · billing correction · other

Justification *

Covering Dr. Meera Nair's inpatients this weekend; need imaging history before today's ward round. Reference: duty roster 25–27 Jul.

Access duration *

☒ 4 hours — single task (default)

☐ 24 hours — covering shift

☐ Care-team addition — ongoing; requires the treating clinician's confirmation

Cancel

Submit request

My access requests

LIFECYCLE · SLA 4 BUSINESS HOURS

REQUESTED	RECORD	REASON	DURATION	STATUS	DETAIL
25 Jul 12:12	MRN- ••••30	Covering clinician	4 h	Pending	SLA 4 business hours · due 14:30 · 🕒 2h 08m remaining — escalates to org privacy officer on breach
25 Jul 09:20	MRN- ••••77	Follow-up call	24 h	Approved	time-boxed · expires in 22:40 · auto-expires approved by privacy lead A. D'Souza 09:44 · every open recorded
24 Jul 17:05	MRN- ••••02	Billing correction	4 h	Denied	Approver reason: "Billing corrections are handled by the billing desk role — raise ticket, clinical access not required." (A. D'Souza, 24 Jul 17:38)
22 Jul 08:15	MRN- ••••19	Covering clinician	24 h	Expired	Auto-expired 23 Jul 08:15 after 3 record opens · all recorded in the access log

STATE C — THE RR-S2 FIX: SAME-ORG PRIORS LOAD FOR TREATMENT, RECORDED NOT REQUESTED

Reading workload > Priya Menon · MRN-004828 · ACC-2026-04471 > Prior studies

Same organisation · same patient · treatment purpose

POLICY AZ-P4

Dr. V. Shetty is reading today's CT Brain for Priya Menon. Priors held anywhere in Meridian Diagnostics for this patient now load directly — the review found RR-S2 wrongly forced a request here, delaying reads.

PRIOR	BRANCH	DATE	REPORT	ACCESS
CT Brain plain ACC-2025-11208	Bandra West	2025-11-30	Dr. S. Iyer — final	Access allowed & recorded — care-team verified · purpose: Treatment Open prior
MRI Brain ACC-2024-07731	Andheri West	2024-06-14	Dr. V. Shetty — final	Access allowed & recorded Open prior
X-ray Sinuses ACC-2023-02114	Powai	2023-02-02	Dr. A. Rao — final	Access allowed & recorded Open prior

🔒 Opening this prior was recorded (DPDP purpose-limited processing — no request needed within your organisation for active treatment).

Approver queue — branch privacy lead

A. D'SOUZA · APPROVER VIEW

REQUESTER	RECORD	REASON · DURATION	DECISION
Dr. V. Shetty Radiologist · identity confirmed ✓ (SSO 12:12)	Fatima Zahra · MRN-004833	Covering clinician · 4 h "Covering Dr. Meera Nair's inpatients this weekend..."	Approve 4 h Deny with reason
K. Verma Admin · identity confirmed ✓	Hiroshi Tanaka · MRN-004777	Billing correction · 4 h	Approve 4 h Deny with reason

Approvers see the full patient identity; requesters do not until approval. Decisions, identities and reasons are recorded in the access log (tier AMBER).

🔒 Access log: 25 Jul 2026 12:10 IST · v.shetty denied open on MRN-••••30 (masked, no identity disclosed) · 12:12 access request AR-2026-0912 submitted (pending, SLA due 14:30) · 12:31 prior ACC-2025-11208 opened under AZ-P4 treatment purpose — recorded, no request required · approver decisions recorded against A. D'Souza.

Why this screen exists: Review found GV-S4's breach runbook "started mid-story" — there was no front door where staff actually create incidents, and PL-S6 misclassified PII-in-application-logs as a SEV3 minor. This screen is the single intake for all incidents, with auto-classification that routes personal-data exposure to the breach path.

Magnus Governance

Meridian Diagnostics · All branches

AR Dr. A. Rao ▼

OVERSIGHT

Incidents2

Breach runbook

Access log

PRIVACY

Deletion requests

Legal holds

Retention policies

Governance > Incidents > Report an incident

Report an incident

Anyone with a Magnus login can report. Classification and clocks are handled after submission — when unsure, report anyway.

STATE — A · INTAKE FORM

What kind of incident? *

Suspected data breach

Patient data exposed, sent to the wrong person, or accessible where it shouldn't be

Security event

Phishing, malware, lost device, suspicious login

Privacy complaint escalation

A GR-S1 complaint that alleges data misuse

System outage with clinical impact

PACS, RIS or reporting down while patients wait

What happened? *

Report share links for signed reports appear to be accessible without OTP when the link is opened from a forwarded WhatsApp message. Noticed on ACC-2026-88214; sampled 4 more accessions with the same behaviour

Do not put patient identifiers here — reference numbers only (MRN, ACC, VIS). This description is visible to responders across teams.

Estimated patients affected

~1,240 (all SMS-link deliveries si

Data types possibly exposed

☒ Contact details

☒ Reports

☐ Images

☐ Payment refs

How was it detected? *

Staff noticed ▼

When first noticed *

2026-07-25 · 18:47 IS

Reporter

Dr. A. Rao — Privacy Officer

Auto-filled; anonymous reporting is not supported — reports are attributed.

Submit incident

Save draft

prototype: alternate states below

STATE — B · AUTO-CLASSIFICATION AFTER SUBMIT

INC-2026-0042 · Classification result

RULES V4 · PRIVACY OFFICER MAY OVERRIDE WITH REASON

BREACH PATH confirmation due in 2 h SEV1 candidate

Classification: PERSONAL DATA BREACH (likely) — exposed report links affect ~1,240 patients. This starts the breach runbook (GV-S4): the CERT-In 6 h clock and DP Board notification clocks begin at **CONFIRMATION**, not intake. Assigned: **Privacy Officer (owner)** + on-call platform SEV1.

WHY THIS CLASS

Data types include reports + contact details; scope > 100 data principals; external accessibility asserted → breach-likely per rule B-03.

NEXT STEP

Owner confirms or rules out within 2 h. Confirmation opens GV-S4 with clocks running; ruling out requires a recorded reason.

PRECEDENT APPLIED

ROUTING

"Patient names found in application logs" (PL-S6 finding)

Routed to **BREACH PATH** — was: SEV3 "minor"; reclassified per review finding. Any personal data outside its intended store is breach-path by default.

Confirm breach — open runbook GV-S4

Rule out (reason required)

Incident triage queue

OWNER: PRIVACY OFFICER · SLA PATTERN PER NS-16

INC REF	CATEGORY	EST. SCOPE	STATUS	CLOCKS	
INC-2026-0042	Suspected data breach Report links open without OTP	~1,240 patients reports + contact details	CONFIRMED BREACH	CERT-In 6 h: 04:12 left DP Board notice: draft due 72 h	Open runbook GV-S4
INC-2026-0039	Security event Suspicious login, billing account	1 staff account no patient data confirmed	INVESTIGATING	triage SLA met	Open
INC-2026-0037	Suspected data breach Patient reported "wrong report" — was own prior study	0 patients	FALSE POSITIVE — closed with reason (recorded)		View
INC-2026-0041	System outage with clinical impact PACS viewer down, Andheri West	Branch 2 STAT reads waiting	MITIGATING also paged branch ops	restore target: 38 m left	Open

🔒 Incident submission, every classification decision (including overrides and rule-outs with reasons), clock starts and runbook openings are recorded in the access log.

Why this screen exists: Review found countdowns on RR-S1, RR-S6, GR-S1, GV-S1, GV-S3 and GV-S4 — but not one of them defined its zero-hour behaviour. This pattern specifies exactly what a countdown does at warning, at breach, and after late resolution, so no timer anywhere in Magnus can expire silently.

Magnus Design System

Meridian Diagnostics · All branches

AR Dr. A. Rao

PATTERNS

Identity confirmation

SLA & escalation

Audit microcopy

Empty & error states

CONFIGURATION

SLA definitions

Escalation routing

Design system > Patterns > SLA breach & escalation

The countdown component — four states, one behaviour

Every SLA timer in Magnus renders this component. States are driven by elapsed % of the SLA window; breach behaviour is never left to the host screen.

STATE — A · THE FOUR COMPONENT STATES

1 · ON TRACK

< 75% ELAPSED

due in 3 h 20 m

Neutral presentation. No row highlight, no notifications. Timer visible wherever the item appears.

2 · WARNING

≥ 75% ELAPSED

due in 48 m

Row gains amber highlight and sorts toward top of its workload. Owner sees an in-app nudge. Still owner's to clear — no escalation yet.

3 · BREACHED

100% — ZERO HOUR

OVERDUE 00:42

Row gains:

Owner: Radiology duty coordinator

Escalated to: Branch medical lead · 21:29 · push + audible

Acknowledge & act

Reassign

Record reason

4 · RESOLVED-LATE

CLOSED AFTER BREACH

closed 1 h 12 m late — reason

A breached item cannot be closed without a reason. Late closures aggregate into the monthly SLA report by branch, SLA type and owner role.

Zero-hour rule.

At breach the item always (1) names an accountable owner, (2) notifies a second, more senior tier, and (3) demands an acknowledgement or a recorded reason. A countdown that reaches zero and merely turns red is a design defect.

----- prototype: alternate states below -----

STATE — B · ESCALATION CONFIG (PER SLA TYPE)

Escalation settings — "Report sign-off (routine)"

ORG ADMIN · IDENTITY-CONFIRMED CHANGES

Warn threshold

75% of SLA window (default)

Breach owner role

Radiology duty coordinator

Resolved to the rostered person at breach time.

Tier-2 escalation delay

30 min after unacknowledged breach

Tier-2: Branch medical lead.

Channels

☒ In-app (always on, cannot be disabled)

☒ Push notification

☒ Audible alert on breach (workstation)

☐ SMS to owner (tier-2 only)

Quiet hours NEVER apply to clinical alerts

— RED-tier critical results and STAT reads escalate with push + audible at any hour. Quiet hours may only suppress non-clinical SLA channels (e.g. billing approvals).

Where the component runs today

ONE CONFIG ROW PER SLA TYPE

SLA	SCREEN	ON BREACH
STAT read overdue (60 min)	RR-S1	Escalate to lead radiologist · push + audible · worklist shows rebalance prompt ("2 unassigned STAT — redistribute?")
Critical result ack lapsed (RED tier, 30 min)	NS-01 / RR-S6	Tier-2 phone call task + audible at referrer-contact desk · quiet hours never apply
Unmatched STAT study (15 min)	RT-S4	Page branch admin · study pinned atop reconciliation queue until claimed
Complaint response overdue (72 h)	GR-S1	Escalate to branch manager · + clinical lead if complaint is care-safety category
Deletion request at day 28 (30-day DPDP window)	GV-S3	Escalate to Privacy Officer + DPO (Priya Sharma) · daily digest until closed or lawfully refused (e.g. legal hold, NS-11)
Billing approval pending 4 h	NS-06	Escalate to org admin · approval card surfaces on admin home

One component, one behaviour

Every countdown in Magnus — clinical, privacy, billing or operational — uses this pattern. Owners are named, escalation is automatic, and late closures carry reasons into the monthly SLA report. **No silent breaches.**

🔒 Breach events, escalation notifications, acknowledgements, reassignments, recorded reasons and every escalation-config change are recorded in the access log.